

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

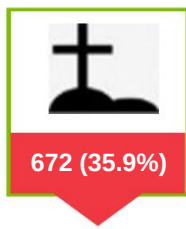
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°057/MVB_10/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (41)	• Haut-Uélé (1/13): Wamba • Ituri (25/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa and Boga. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Makiso, Lubunga and Mangobo
Reporting date	July 10, 2026
Publication date	July 11, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



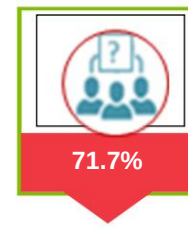
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Over the past 24 hours, **43 new confirmed cases and 24 deaths (62.5% community transmission) have been recorded** in the provinces of Ituri (42 cases and 24 deaths) and North Kivu (1 case), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at **1,873** confirmed cases and **672** deaths, representing an overall case fatality rate of **35.9%**.

- **Nine (9)** deaths were recorded among patients with Ebola virus disease in the CTE/CT in Ituri (3 at CTE CME, 2 at CTE Elikya, 1 at CTE Bunia, 1 at CTE ISTM, 1 at CT Sota and 1 at CT Nizi).
- **Six (6)** patients with Ebola virus disease in Ituri province (4 at CTE Bunia, 1 at CTE Mongbwalu and 1 at CTE ISTM) have been declared cured after obtaining negative control tests.
- **Inauguration of the CTE extension at the HGR Bunia and CT of the Kingoze displaced persons camp in Ituri** by His Excellency the Minister of Public Health, Hygiene and Social Welfare, accompanied by the Secretary General for Health, members of the national coordination as well as technical and financial partners.

The **epidemic has now spread to Tshopo and Haut-Uele**. Although current investigations suggest that all cases detected in these two provinces were essentially imported from Niania in Ituri, it is necessary and appropriate, given their strategic locations and especially the presence of numerous risk factors for large-scale spread, including the ongoing challenges of managing Ebola in these two provinces, to consider these two provinces as epidemic zones.

in order to enable the implementation of appropriate response measures and to prevent any avoidable outbreaks of cases

- Reception in Bunia of 200 secure blood bags transported from Kinshasa by the program national blood transfusion service, including 135 O+, 27 A+, 34 B+, 2 O⁻ and 2 AB+.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on **May 15, 2026**, the Ebola virus disease epidemic caused by the **Bundibugyo ebolavirus**

The outbreak continues to evolve within a complex humanitarian and security context. Initially detected in the **Mongbwalu** Health Zone (Ituri), it has spread to **three provinces (Ituri, North Kivu, and South Kivu)**, affecting **37 health zones**. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan facilitate transmission and complicate response operations. Multisectoral coordination continues to limit the spread of the epidemic and strengthen public health interventions.

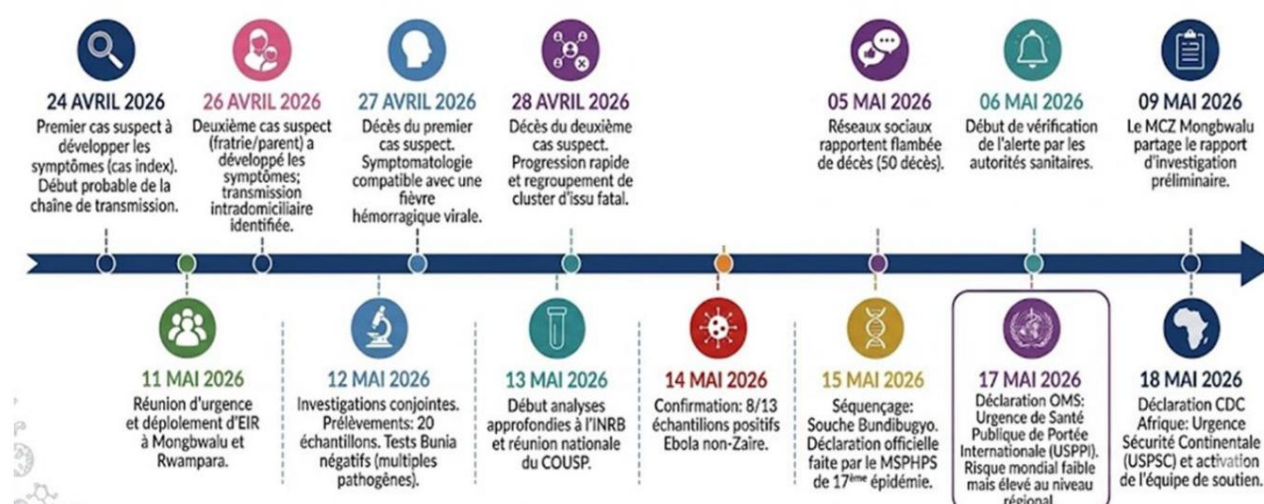


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 10, 2026

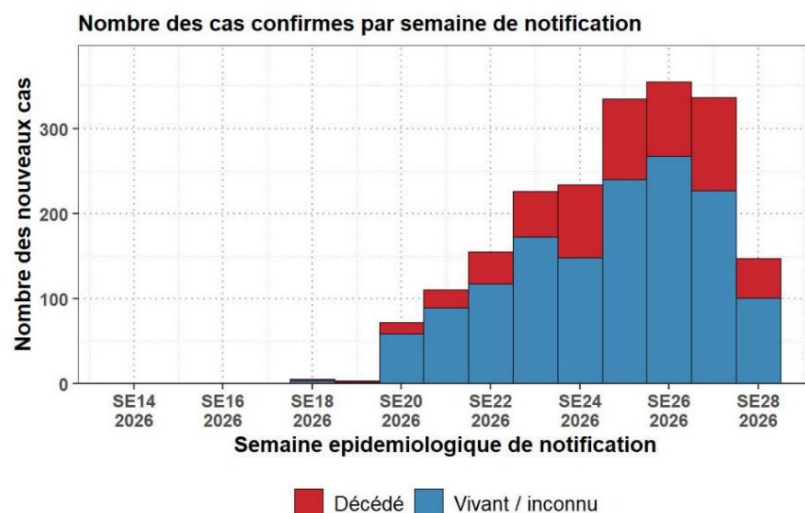
Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New cases confirmed (24h)
Ituri*	1705	577*	33.8%	25 out of 36 (69.4%)	42
North Kivu	165	94	57.0%	11 out of 34 (32.4%)	1
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Haut-Uélé**	1	1	100.0%	1 out of 13 (7.7%)	0
Tshopo**	3 out of 23 (13.0%)	2	50.0%		0
Total 1,873,672.41 out of 140 *9 deaths in Ituri were recorded among 131.9% with Ebola virus disease (previously confirmed cases) in Ebola treatment centers. ** : Cases counted in Niania not already included to avoid double counting					43

At the end of the day on July 10, 2026, no new health zones had been affected in the provinces of Ituri, North Kivu, and South Kivu in the past 24 hours. However, with the inclusion of the provinces of Haut-Uélé and Tshopo—whose previous confirmed cases are epidemiologically linked to the epicenter—

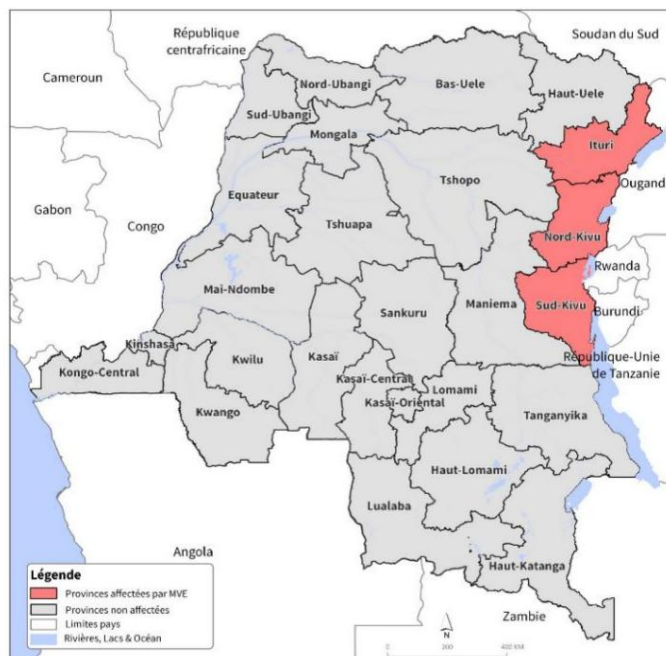
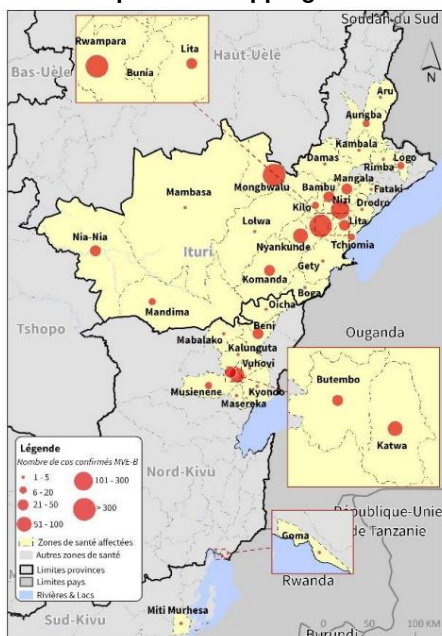
of Ituri (Nia-Nia/PK51) — the cumulative number of affected health zones now stands at 41 out of 140 (Tshopo provincial denominator currently being consolidated; section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission. As of July 7, 2026, South Kivu province officially passed the 42-day mark without notification of a new confirmed case, which corresponds to twice the maximum incubation period of the virus.

3.2 Time-Based Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) confirmed continuous and increased community transmission in week 25. Compared to all weeks in which cases have been recorded since the beginning of the epidemic, weeks 25, 26, and 27...

continue to see growth in the number of reported cases, with more than 300 cases per week.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 10, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 10, 2026)

Province / Health Zone	Cumulative confirmed cases	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1705	577	33.8%
Bunia	503	156	31.0%
Rwampara	384	89	23.2%
Mongbwalu	329	171	52.0%
Nyankunde	97	23	23.7%
Nizi	110	36	32.7%
Lita	45	19	42.2%
Komanda	25	8	32.0%
Bamboo	31	10	32.3%
Kilo	9	1	11.1%
Mangala	50	29	58.0%
Tchomia	19	4	21.1%
Damascus	5	0	0.0%
Aungba	6	2	33.3%
Rimba	4	0	0.0%
Aru	3		33.3%
Mambasa	5	1	80.0%
Logo	7	4	0.0%
Drodro	4	0	75.0%
Gety		3	0.0%
Kambala	1	0	100.0%
Nia-Nia	2	2	37.1%
Fataki	35 4	13 1	25.0%
Mandima	8	3	37.5%
Lolwa			100.0%
Boga	11	11	100.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	165	94	57.0%
Butembo	43	20	46.5%
Katwa	67	45	67.2%
Blessed	30	20	66.7%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	5	1	60.0%
Goma		3	0.0%
Masereka	1	0	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
Musienene	3	1	22.2%
SOUTH KIVU	1193	021	33.3%
Miti-Murhesa	3	1	33.3%
HAUT-UÉLÉ*	1	1	100.0%

Wamba	1	1	100.0%
TSHOPO*	4	2	50.0%
Makiso-Kisangani	2	2	100.0%
Mangobo	1	0	0.0%
Lubunga	1	0	0.0%
TOTAL	1873	672	35.9%

* : Not counted as already included in the cases of the Niania Health Zone.

The provinces of Haut-Uélé and Tshopo are included in this report for the first time. The cases there are all epidemiologically linked to the epicenter in Ituri (Nia-Nia health zone / PK51 mining site), through contacts and population movements towards Kisangani and Haut-Uélé.

Haut-Uélé: One (1) case confirmed by the INRB Kinshasa has been recorded in the Wamba health zone, with one (1) death (100% case fatality rate). This case, linked to importation from Nia-Nia, is counted under Haut-Uélé. The health zones of Boma Mangbetu, Pawa and Isiro have reported suspected cases for which certification by INRB Kinshasa is pending; they are not counted among the confirmed cases.

Tshopo: The province reported four (4) confirmed cases at the provincial level. The confirmed case in Ituri (Nia-Nia) involved an individual who escaped from the Nia-Nia General Referral Hospital and was found on July 7, 2026, in the Lubunga health zone (Kisangani). The three (3) remaining cases are attributed to Tshopo according to their health zone of detection: Makiso-Kisangani (two confirmed cases, two deaths – confirmed post-mortem after the bodies were transferred to the REKAPI morgue in Kisangani) and Mangobo (one confirmed case). Tshopo thus has four (4) confirmed cases and two (2) deaths. These cases are still counted under Nia-Nia/Ituri to avoid double counting.

Over the past 24 hours, **43 new confirmed cases and 24 deaths (62.5% community transmission) have been recorded** in the provinces of Ituri (42 cases and 24 deaths) and North Kivu (1 case), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo. The cumulative total now stands at 1,873 confirmed cases and 672 deaths, representing an overall case fatality rate of **35.9%**.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **91.0%** of cases (n=1,705) and **85.9%** of deaths (n=577), representing a crude case fatality rate of **33.8%**, making it the epicenter of the epidemic. The health zones of Bunia (503 cases), Rwampara (384 cases), Mongbwalu (329 cases), Nizi (110 cases), and Nyankunde (97 cases) remain the most affected, accounting for approximately 83.5% of reported cases at the provincial level and 76.0% at the national level. In Nizi, the epidemic is currently progressing more rapidly than in the other zones.

Severity of the epidemic in North Kivu

A total of 165 confirmed cases and 94 deaths have been reported in North Kivu province since the start of the outbreak, representing a **crude case fatality rate of 57.0%**. This high case fatality rate is concentrated in the health zones of **Katwa** (67.2%), **Beni** (66.7%), and **Butembo** (46.5%), while the other health zones account for only 36.0% of the case fatality rate. The three health zones with the highest number of reported cases are Katwa (n=67), followed by Butembo (n=43) and Beni (n=30). A case fatality rate of 100% was observed in the exceptionally small Vuhovi health zone.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Inauguration of the CTE extension at the HGR Bunia and CT of the displaced persons camp of Kingozi in Ituri by His Excellency the Minister of Public Health, Hygiene and Social Welfare, accompanied by the Secretary General for Health, members of the national coordination as well as technical and financial partners.
- Delivery of 200 bags of blood from PNTS Kinshasa.
- Continuation of on-the-ground supervision activities of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 10, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)			0	19
New Alerts		0	8	1497
Total alerts for the day		852	8	1516
Alerts investigated		852	8	1285
Investigation rate		852	100.0% 5	84.8%
Alerts confirmed	Alive		0	208
	Deceased	19,637,656,425 64.8% 160.6% 43 26		91
Total suspected cases for today	225	69	5	299

During the day of July 10, 2026, a total of 1,516 alerts were recorded in the provinces of Ituri, North Kivu and South Kivu, including 1,497 new alerts and 19 alerts reported from the previous day, demonstrating the maintenance of active epidemiological surveillance in the affected provinces.

The majority of alerts were reported in North Kivu, with 852 alerts, or 56.2%, compared to 656 alerts, or 43.3%, in Ituri and 0.5% in South Kivu.

Of the alerts recorded, 1,285 were investigated, representing an overall investigation rate of 84.8%. The investigation rate was 64.8% in Ituri and 100% in the other two provinces, highlighting the need to strengthen investigative capacities in Ituri province.

The investigations led to the validation of 299 suspected cases, including 208 living individuals (69.6%) and 91 deaths (30.4%). Ituri recorded the highest number of validated suspected cases, with 225 (75.3%), compared to 69 (23.1%) in North Kivu and 1.8% in South Kivu.

Table 4. Contact tracing of confirmed cases (1) as of July 10, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	11,480	7,817	68.1%
North Kivu	2,280	2045	89.7%
South Kivu*	0	0	N / A
Total	13,760	9,862	71.7%

* End of follow-up for all contacts listed in the province of South Kivu.

As of July 10, 13,760 contacts were under surveillance in the provinces of Ituri and North Kivu. Of these, 9,862 were actually seen, representing an overall follow-up rate of 71.7%. Ituri had the highest number of contacts.

A large number of contacts were under follow-up: 11,480 (83.4% of the total), with 7,817 contacts seen, representing a follow-up rate of 68.1%. In North Kivu, 2,280 contacts were under monitoring, of which 2,045 (89.7%) were visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 10, 2026

Province	Number of new contacts		Number of contacts had become suspects		Number of contacts outings every 21 days	
	n		n		n	%
Ituri		78.5 %	8	66.7 %		38.1%
North Kivu		21.5%	4	33.3%		61.9%
Total	543,149,692	100.0%	12	100.0%	69,112,181	100.0%

The day ended on July 10th with **692 new contacts** listed, the majority of them in Ituri (543) (78.5%), while North Kivu listed 149 (21.5%). Among the contacts monitored, twelve (12) developed symptoms consistent with the definition of a suspected case, including eight (8) in Ituri (66.7%) and four (4) in North Kivu (33.3%). These contacts require immediate investigation and management in accordance with current procedures.

In addition, 181 contacts completed their 21-day follow-up period without developing symptoms consistent with the disease, including 69 in Ituri, or 38.1%, and 112 in North Kivu, or 61.9%.

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure early detection of secondary cases and to quickly interrupt any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 10, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100% (57/57)	100.0% (14/14)	100.0% (71/71)
Number of people who went through PoE/PoC testing,	108,402	56,464	164,866
% of travelers screened, % of	92.8%	98.0%	94.6%
travelers who washed their hands, % of	93.1%	98.0%	94.8%
travelers made aware of the issue	98.9%	98.0%	98.6%
Number of alerts notified	5	2	7
Number of validated live alerts	2	2	4
Number of lifeless bodies intercepted today	0	2	2
Number of problematic contacts intercepted today; %			0
of validated live alerts referred to CT/CTE; % of	0 100%	0	50%
deceased bodies swabbed	NA	0%	0%
Number of positive results of referred suspected cases	0	0% 0	0

4.3. Laboratory

- **Ituri** : 177 samples collected and analyzed (**positivity 23.7%; n=42**).
- **North Kivu** : 54 samples collected and analyzed (**positivity 1.9%; n=1**).
- **South Kivu** : 5 samples collected and currently being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: Thirty (30) confirmed cases were addressed through ringside activities. Thirty-one (31) households were identified, of which 29 were decontaminated (93.5%). Eight (8) healthcare facilities were identified, of which seven (7) were decontaminated. In addition, 20 IPC/WASH assessments, 65 IPC/EDS kits or supplies were distributed, 31 community health workers were monitored and supported, three (3) community health workers were decontaminated, and 24 vehicles were decontaminated. Eighteen (18) public health workers were trained, and 86 service providers were briefed during formative supervision sessions.

North Kivu: Conclusion of the briefing for 22 healthcare providers on IPC, CREC, and SCBC in the Butembo Health Zone, with support from CARITAS; Training of 15 IPC providers from 5 health facilities in the Beni Health Zone, with support from IRC. Implementation of the Scorecard assessment in the Beni (3 health facilities), Kibua (1 health facility), Musienene (2 health facilities), and Oïcha (5 health facilities), revealing an average score of 35%; Training of 15 providers from CH La Vérité on IPC/WASH, with support from WHO. Monitoring and support of 81 health facilities, including 27 in Oïcha, 24 in Butembo, 10 in Musienene, 1 in Kyondo, and 2 traditional healer practices in Beni. Risk assessment of exposure for 16 persons deprived of their liberty (PPL): 10 in Butembo, including 5 at high risk and 3 at low risk, and 6 in Musienene, including 4 at high risk and 2 at low risk. Investigation of 6 expected death alerts in Musienene, of which 1 case was swabbed and secured, as well as 3 alerts in Beni. Briefing of 488 service providers during formative supervision, distributed as follows: 195 in Beni, 114 in Oïcha, 79 in Butembo, 44 in Kyondo and 26 in Musienene; Decontamination of 21 sites, including 3 health facilities (ESS) in Musienene (including 1 traditional healthcare structure) and 18 places of residence in Butembo, notably around a confirmed case treated at the Butembo University Hospital Center (CUG); Conducting assessments in 42 priority health facilities in the health zones of Butembo, Katwa, Beni, Kyondo, Karisimbi, Musienene and Goma.

The decontamination activities in the Musienene health zone concerned the Kavale Hospital Center, the Sainte Rita Hospital Center and the Kiheri Traditional Care Dispensary.

A briefing of 156 healthcare providers on the main themes of IPC took place during the supervision and support of priority structures.

4.5 Holistic care

Ituri:

- Nine (9) deaths were recorded among patients with Ebola virus disease in the CTE/CT in Ituri (3 at CTE CME, 2 at CTE Elikya, 1 at CTE Bunia, 1 at CTE ISTM, 1 at CT Sota and 1 at CT Nizi).
- Six (6) patients with Ebola virus disease in Ituri province (4 at CTE Bunia, 1 at CTE Mongwalu and 1 at CTE ISTM) have been declared cured after obtaining negative control tests.
- Receipt of 16 cool boxes containing 200 bags of blood (135 O+, 27 A+, 34 B+, 2 O- and 2 AB+) from Kinshasa to the national blood transfusion program.
- By the end of the day on July 10, 981 hot meals had been served: 160 to suspected cases, 178 to Confirmed cases: 586 in PPLs and 57 in accompanying persons.

Patient movement:

- 110 admissions were recorded in the CTE/CT/CI facilities of the three provinces, including 69 in Ituri, 36 in North Kivu and 5 in South Kivu, according to the latest available data. Discharges totaled 126 patients.

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 10, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	535	225	19	779
Total admissions (24h)	69	36	5	110
Exits (24h)				
Deceased (Suspects/Confessions)	18	0	0	18
No case	28	64	0	92
Healed	6	0	0	6
Escapees (Suspects/Confessions)	9	0	0	9
Transferred to the HGR	0	1	0	1
Total outflows	61	65	0	126
Patients in isolation (End of Day),	543	196	24	763
including confirmed cases	237	22	0	259
(NC+AC) and suspected cases	306	174	24	504
Overall occupancy rate	88%	135.2%	53.3%	95.1%

In total, **763** patients were in isolation, including 259 confirmed cases and 504 suspected cases, for an overall bed occupancy rate of **95.1%**.

Table 8. Key indicators of mental health and psychosocial support activities as of July 10, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	1 (12.5%)	0.0%	0
Confirmed cases currently being monitored that have received interventions SMSPS	105 (58.6%)	7 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	11 (27.5%)	8 (100.0%)	2
Suspected cases under follow-up that have received interventions SMSPS	122 (77.7%)	16 (100.0%)	2 (100.0%)
Laboratory results have been announced.	14	2	1
Including positive ones	0	0	0
Number of children separated in daycare who benefited from SMSPS interventions	10	1	0
Number of children separated in the community who have benefited from interventions SMSPS	0	0	7
Number of accompanying persons at the CTE who benefited from SMSPS interventions	110	24	0
Number of PPLs who benefited from SMSPS interventions	43	0	0
Number of household and community members who benefited from SMSPS interventions	62	0	0
Number of EDS supported	31	0	0

4.6 Continuity of care

- In total, **58 PPLs** have been confirmed in the province of Ituri since the start of the epidemic.

4.7 Risk communication and community engagement

Ituri: One (1) person was engaged during an advocacy meeting; 2,510 people were reached during home visits (1,255 men and 1,255 women); 1,846 people were reached during educational talks and focus groups (361 men and 1,485 women); 7,697 people were reached during 30 mass awareness campaigns (2,854 men and 4,843 women); and 2,391 people were sensitized in churches, mosques and markets.

Community feedback: Fifty-eight (58) feedback items and informational issues were reported, and 11 were clarified, representing 17% according to the consolidated presentation. The main concerns related to the availability of medications and vaccines, clinical trials, investigation timelines, waste management, and the presentation of bodies to families.

Support for the pillars of the response

Surveillance: 50 community alerts were reported, including 35 live alerts and 15 deaths, in the health zones of Nizi and Bunia.

CREC: Twenty-two (22) IT and ITA from the Nia-Nia Health Zone and 14 Red Cross volunteers were briefed on Ebola Virus Disease (EVD); two radio programs and six public service announcements were produced and broadcast, 36 community radio stations were engaged, and 15 posters and IEC materials were distributed.

North Kivu: Community mobilization and outreach: 197 community actors reported their activities; 4,391 households were visited; 15,679 people were reached with prevention messages during door-to-door visits; 77 people were reached through dialogues and educational talks; and 112 people were sensitized in public places. Twenty-seven (27) radio stations broadcast at least one media segment on Ebola Virus Disease (EVD).

Community surveillance and feedback management: 133 alerts of suspected cases were reported by community actors; three (3) out of five feedback or infodemics were captured and resolved within 48 hours; and 1,115 people were engaged in community actions. Briefings were organized for 41 managers of private laboratories and traditional healers in Beni, 89 members of Community Action Committees (CACs) in Mabalako, 50 community actors in Kirotshe, and 47 community actors in Walikale.

Specific feedback: In the Beni health zone, community members questioned the continued involvement of actors who participated in the 2018 response. Another concern was the announcement of a positive outcome after a body was returned to the family by a private facility. The actions adopted include explaining how to build upon past experience and strengthening collaboration between private medical centers, the health zone coordination team, and the key stakeholders in the response.

4.8. Logistics

Ituri

- Receipt of 200 units of blood from the National Blood Transfusion Program and delivery of IPC/EDS supplies to the health zones of Lita, Ariwara, Logo, Mongbwalu, and Mangala, as well as to the Bunia General Referral Hospital (HGR) treatment center. A total of 25 vehicles and five (5) ambulances were made available to the various stakeholders. A total of 295 liters of fuel was distributed to the ambulances and operational vehicles.

North Kivu

- Unpacking and distribution of inputs and medicines received from UNICEF and UG-PDSS; delivery of PCI/EDS inputs to the health zones of Mabalako, Masereka, Kalunguta and Katwa as well as to the INRB; receipt of 1,800 liters of diesel, of which 360 liters were allocated to the INRB; deployment of a tent and its accessories in Katwa; and delivery of 42 motorcycles intended for CREC and community surveillance in the health zones of Beni (8), Kalunguta (4), Oicha (4), Mabalako (2), Butembo (8), Katwa (9), Musienene (4), Kyondo (2) and Masereka (1).

4.9. Security

- The Avakubi PoC, in the Bafwanduo AS of the Nia-Nia Health Zone, was vandalized by unidentified individuals. At the Mudzibala PoC, threats were reported against service providers after the transfer of an adolescent to the CTE, while some humanitarian and military personnel refused screening and handwashing.
- The Chief Medical Officer of the Nia-Nia Health Zone pleaded for the securing of the PK 51 locality, where a health center was ransacked for the second time, in order to allow the resumption of response activities and dignified and safe burials.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Continued community awareness-raising on the prevention of sexual exploitation and abuse.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, strengthen team security, and increase the number of EDS teams.
2 Insufficient capacity of CTE/CT facilities and saturation in North Kivu (135.2%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures, and improve laboratory turnaround times.
3 Contact tracking performance not optimal (71.7% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation, prune any possible duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.) Active field verification
6 Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (112 cases) HCW (including 35 deaths, case fatality rate 31.2%)	Urgent supply, strengthening of IPC training
8. Sample stock not yet analyzed (backlog) in the laboratory at North Kivu and late diagnosis	Delayed confirmation, high lethality (57.3%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities, improve the flow of laboratory data
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10 Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11. Insufficient resources	Limitation of the implementation	Urgent mobilization of resources, advocacy with

financial (gap of 20 million USD)	work of all the pillars of the response	partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (non-Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Inauguration of the extension of a CTE at the Bunia General Referral Hospital and CT of the Kigonze displaced persons camp in I



Closing of the training for 20 midwives on reproductive health



Receipt of 200 bags of blood from PNTS

**POUR TOUTE INFORMATION
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**Avec l'appui technique de l'Organisation Mondiale
de la Santé (OMS), de Africa CDC et Bluesquare**